

HOUSE No. 2510

The Commonwealth of Massachusetts

PRESENTED BY:

Edward R. Philips

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to improve outcomes for sudden cardiac arrest.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Edward R. Philips</i>	<i>8th Norfolk</i>	<i>1/16/2025</i>
<i>Paul McMurtry</i>	<i>11th Norfolk</i>	<i>10/13/2025</i>
<i>Richard G. Wells, Jr.</i>	<i>7th Norfolk</i>	<i>10/13/2025</i>
<i>Patrick Joseph Kearney</i>	<i>4th Plymouth</i>	<i>10/13/2025</i>
<i>Hannah Bowen</i>	<i>6th Essex</i>	<i>10/24/2025</i>

HOUSE No. 2510

By Representative Philips of Sharon, a petition (accompanied by bill, House, No. 2510) of Edward R. Philips for legislation to improve outcomes for sudden cardiac arrest by providing additional safeguards and resources. Public Health.

[SIMILAR MATTER FILED IN PREVIOUS SESSION
SEE HOUSE, NO. 2250 OF 2023-2024.]

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to improve outcomes for sudden cardiac arrest.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Subsection (b) of section 18B of chapter 6A, as appearing in the 2022
2 Official Edition, is hereby amended by striking out the figure "13" and inserting in place thereof
3 the following figure:- "15", and;

4 Said subsection is hereby further amended by inserting after the words "Massachusetts
5 Ambulance Association, Inc.", in line 42, the following words:- " , 1 of whom shall be a
6 physician with a specialty in emergency medicine," and;

7 Said subsection is hereby further amended by striking out, in lines 42 through 44, the
8 words "and 1 of whom shall be a manager or supervisor of a PSAP and a nominated

representative of the Massachusetts Communication Supervisors Association, Inc.” and inserting in place thereof the following words:-

“1 of whom shall be a manager or supervisor of a PSAP serving a population of less than 50,000 people and a nominated representative of the Massachusetts Communication Supervisors Association, Inc. and 1 of whom shall be a manager or supervisor of a PSAP serving a population of more than 50,000 people and a nominated representative of the Massachusetts Communication Supervisors Association, Inc..”

SECTION 2. Subsection (g) of said section 18B of said chapter 6A, as so appearing, is hereby amended by adding the following paragraphs:-

The State 911 Department is hereby directed, pursuant to the provisions of chapter 30A of the General Laws, to adopt, amend, and enforce regulations to ensure ongoing Quality assurance and Improvement (QA/QI) for all emergency calls in which a cardiac arrest is confirmed by Emergency Medical Services (EMS) personnel and resuscitation is attempted. The regulations shall include, but are not be limited to, the following requirements:

a) Audit of Dispatch Calls: Within 60 days following the date of receipt of all 911 or emergency calls received at any PSAP in which resuscitation for cardiac arrest is indicated or attempted or 60 days from the date on which an EMS agency notifies a PSAP of a qualifying incident, whichever is shorter, the dispatch call shall undergo a review and assessment for Quality Assurance and Improvement purposes. An analysis and explanation for reasons for the non-recognition of cardiac arrest and any delays shall be documented as part of the assessment. PSAPs shall submit all complete reviews to the State 911 Department within 10 days of completing the Quality Assurance and Improvement review in a form and manner prescribed by

the State 911 Department. The State 911 Department shall develop an electronic portal system to accept submissions of qualifying calls from PSAPs and provide status and feedback regarding the Department's review.

b) Data Collection: The call taking and Quality Assurance and Improvement process must collect key data, as defined by the department, including, but limited to, time intervals from call receipt to "hands on chest", attempted 911 directed or assisted defibrillation, and EMS arrival.

c) Individual Review: Each cardiac arrest call shall undergo an individual Quality Assurance and Improvement review by the PSAP Director or a designated Quality Assurance and Improvement professional, and shall include appropriate feedback and recommendations .

d) Review by the State 911 Department: The State 911 Department shall conduct a review of all reported cardiac arrest calls in accordance with AEMPDS certification standards.

e) The State 911 Department shall develop remediation and enforcement policies to address PSAP non-compliance.

f) Annual Reporting: The State 911 Department shall compile, summarize and make publicly available on a Statewide and PSAP level, on their website Quality Assurance and Improvement reports annually. The reports shall highlight trends and areas for improvement. PSAPs shall be listed by name.

g) Training and Improvement: Quality Assurance and Improvement Reports shall be used to identify and address training needs for emergency dispatch personnel and ensure PSAP compliance with regulatory requirements to provide telephone, 911-assisted CPR, and

Emergency Medical Dispatch. The department shall establish, and from time to time amend, a list of required trainings and implement appropriate mechanisms to ensure compliance with training requirements.

The Department of Public Health shall promulgate regulations to require that all EMS agencies report all instances of cardiac arrest, including those where resuscitation is attempted, to the State 911 Department. This reporting shall include relevant data as determined necessary for Quality Assurance and Improvement and statistical analysis, including but not limited, to patient condition upon first contact with EMS, number of defibrillation attempts, whether or not bystander CPR was in progress, patient status upon arrival at the definitive care facility/ER if transported or indication of pronouncement on scene, if applicable. All reports of incidents reported by EMS agencies shall be forwarded to the PSAP of jurisdiction and, when available, be published in the required portal system.

The State 911 Department, in collaboration with the Department of Public Health, shall ensure that data collection and reporting protocols respect the privacy and dignity of individuals and comply with applicable laws and regulations.

The State 911 Department shall develop an annual award for up to 3 individuals employed by a PSAP for superior performance of duties that substantially contributed to the survival of victims of out-of-hospital cardiac arrests in the Commonwealth. The State 911 Department shall further develop an annual award to be awarded to the PSAP on the basis of high performance and contribution to the survival of victims of out-of-hospital cardiac arrests in the Commonwealth.

SECTION 3. Section 12V ½ of chapter 112, as so appearing, is hereby amended by adding the following in subsections:-

(d) Notwithstanding any general or special law to the contrary, an AED registry shall be established for the purpose of allowing local 911 telecommunicator to locate accessible AEDs.

Any AED used in a public access defibrillation program shall register the device through the PSAP medical control director (MCD).

(e) Notwithstanding any general or special law to the contrary, signage located throughout buildings shall clearly indicate AED locations.

a) Monitor performance in terms of both success and failure, identify problems, and track progress.

b) Enhance performance of EMS systems with emphasis on dispatcher-assisted CPR and high-performance CPR.

c) Develop strategies to improve systems of care within hospital settings and special resuscitation circumstances.

d) Expand basic, clinical, translational, and health services research in cardiac arrest resuscitation and promote innovative technologies and treatments.

e) Educate and train the public in CPR, use of automated external defibrillators, and EMS-system activation

SECTION 5. The State 911 Department and the Department of Public Health shall report to the Joint Committee on Public Health and Public Safety regarding the implementation of

93 regulations under this section and their impact on emergency response to cardiac arrest cases not
94 later than one year after the promulgation of said regulations.

95 SECTION 6: This act shall take effect 180 days after its passage.